

إِسْم الْمَرِيضِ رُبَاعِيًّا : مَنِي عَلِي صَبَاح عَمِيرَه

الحي / المدينة : العياط

المحافضة : الحيزه

رقم البطاقة : 28804032101507 رقم التلفون : 01129410048

العمر(تاريخ الميلاد) : 03-04-1988 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج : الاورام

خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقع:

التوقيع:

التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د.ا ----- / د.ا

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جہۃ العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: مني علي صباح عمره National ID: 28804032101507	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

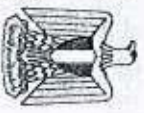
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 وزارة الداخلية
 مصلحة الترخيص
 ملى
 على صباح عبيره
 قرية بونفت
 مركز العياط - الجيزة
 ٢٨٨٠٤٠٣٢١٠١٥٠٧
 KT3017314


 ٢٠٢٤/٠٩ ٢٨٨٠٤٠٣٢١٠١٥٠٧
 حاصلة على دبلوم صنيع
 أنش
 مسجلة
 محرم عيد محمود عيد
 البطاقة سارية حتى ٢٠٣١/٠٩/٠٩



ك
 م
 د

٢٨٨٠٤٠٣٢١٠١٥٠٧



Ministry of Health and Family Welfare, Government of India

National Programme
on Cancer Control

An Patient ID

1- Name *Ashwini Prasad*

Sex

MDT form

2- Medical number

Female

83239462

Q13 Med

Date 3762-2024

3- ID number 28804032101502

Patient Age 27 Y



Family history

Phone

01145410048

BMI

25

REASON OF MDT

Marital status

TARS Score

1

GROUP

Date of MDT

2025-05-1

Co morbidity

Menopausal
Status

T0 T1 N1 STAGE

Clinical data

Breast Imaging :

RT USG multi-focal 20x20 multiple LFs

Mets Workup :

pausifree

BX

ER

PR

Her 2 Neu

STSH

Ki67

IDC

0/9

5/8

1

15

T

N

M

-d

1

0

Sonomammography

PATIENT HISTORY

received NACT TCHP X6 last 23/3/2025 with complete clinical response

RT MRM :free margin

LN 0/4

free nipple / areola

YPT0N0

MDT OUTCOME & PLAN

ADJUVANT PHESGO X11

PORT

ADJUVANT TAM

MEDICAL ONCOLOGY

SIGNATURE

RADIOLOGY

SURGERY

PATHOLOGY

Psychiatric

Nutrition

Name:	مني علي صباح	
MR. :	32	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 37-year-old individual with a history of right-sided invasive ductal carcinoma of the breast, estrogen-receptor negative (ER 0/8), progesterone-receptor positive (PR 5/8), HER2-positive (3+), Ki-67 15%, clinical stage T4, N1, M0. Neoadjuvant chemotherapy with the TCHP regimen was administered for six cycles, completed on 23 March 2025, resulting in a complete clinical response. Subsequent right-sided modified radical mastectomy demonstrated free margins, 0/4 positive lymph nodes, and an intact nipple-areolar complex. Adjuvant therapy included PHESGO for 11 cycles, adjuvant radiotherapy, and Zoladex (goserelin) initiated on 28 January 2026. Current medications are PHESGO 600 mg subcutaneously, tamoxifen 20 mg daily, calcium 1 tablet daily, and Zoladex 10.8 mg administered on 4 April 2026. Recent investigations show a normal chest X-ray, normal serum markers, and an echocardiogram on 28 January 2026 with an ejection fraction of 69%. The patient has completed eleven cycles of PHESGO to date. The patient presented with a two-month history of right hip joint pain. Imaging with a new bone densitometry (DEXA) and targeted ultrasound (TVUS) was ordered to evaluate for possible bone involvement related to tamoxifen therapy. Zoladex was continued, with the next dose due on 5 April 2026. On 31 May 2026, follow-up imaging demonstrated a stable finding with a thickness of 4 mm on the most recent TVUS performed on 18 February 2026. The patient remains on Zometa, tamoxifen, calcium, and vitamin D supplementation.		
تحريراً في ,		
	الوقت :	التاريخ : 21-09-2026
	dina makram	الطبيب المعالج :
يعتمد ,		

Patient Name : الأستاذة / منى على صباح عميره	Request No. : 13008436
Patient No. : 28804032101507	Request Date : 20/04/2025
Path No. : 1415/25	Verification Date : 05/05/2025
Sex : Female	Financial Category : أورام دار السلام - مبادره
Age : 36 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegezy

Pathology Report

Clinical Data: Right breast cancer post NACT .

Nature of the specimen: Right breast & right axilla (MRM)

Gross Examination:

- Procedure: Right MRM
- measured 20x15x2 cm with covering skin ellipse measured 19x7 cm .
- Tumor extent : A greyish white tumour bed site
- Size: 10.5 x9.5 x2 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 0.2 cm away from deep margin
- Axillary lymph nodes: Fibrofatty tissue piece measured 6x5 cm , careful dissection revealed entangling 4 nodules only up to 2.3x1 cm

Block designation :

- 1>10.Rubbery area
- 11.Inked deep margin
- 12.Nipple
- 13.LNS

Grossing by : Dr/Yasmine Essam

- Microscopic examination:
- Histological type: No remnants of tumor
- Ductal carcinoma in situ: Free
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/4)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic complete response

Diagnosis:

Right MRM (post NACT) :

Verified by :

Dr.Heba Hasan



Patient Name :	الأستاذة / منى على صباح عميره	Request No. :	13008436
Patient No. :	28804032101507	Request Date :	20/04/2025
Ph No. :	1415/25	Verification Date :	05/05/2025
Sex :	Female	Financial Category :	اورام دار السلام - مبادره
Age :	36 Y	Department :	Out Patient
Referring Doc :		Registered by :	Mrs.Mayada Hegazy

Pathology Report

no remenants of tumor tissue
Free margins
Free nipple and areola
Free L.Ns (0/4)
Pathologic complete response
Stage : ypT0 , ypN0

Signature
Prof. Dr. Gamal Abdelatty
Dr.Menan Hosam

Verified by :
Dr.Heba Hasan





Nuclear Medicine Unit

Patient Name: Mona Ali Sabah
Referring physician: Prof.
Date: 17/11/2024

^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No scintigraphic evidence of bony metastases .
- Heterogenous abnormal radiotracer uptake is seen at the spine , more determined at mid dorsal , upper lumbar vertebrae likely of Spondylodegenerative changes ,
- Mild osteoarthritic changes are seen at both shoulders , both knees .
- Both kidneys are normally seen.

Conclusion:

No definite hot spot that could account for osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR. Shorin Mahdy, MD

DR/ Mohamed Emam

Patient Name	: الأستاذة / منى على صباح عميرة	Request No.	: 13008436
Patient No.	: 28804032101507	Request Date	: 20/04/2025
Path No.	: 1415/25	Verification Date	: 05/05/2025
Sex	: Female	Financial Category	: نورام دار السلام - مياره
Age	: 36 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer post NACT .
Nature of the specimen: Right breast & right axilla (MRM)

Gross Examination:

- Procedure: Right MRM
- measured 20x15x2 cm with covering skin ellipse measured 19x7 cm .
- Tumor extent : A greyish white tumour bed site
- Size: 10.5 x9.5 x2 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 0.2 cm away from deep margin
- Axillary lymph nodes: Fibrofatty tissue piece measured 6x5 cm , careful dissection revealed entangling 4 nodules only up to 2.3x1 cm

Block designation :

- 1>10. Rubbery area
11. Inked deep margin
12. Nipple
13. LNS

Grossing by : Dr/Yasmine Essam

- Microscopic examination:
- Histological type: No remnants of tumor
- Ductal carcinoma in situ: Free
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/4)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic complete response

Diagnosis:

Right MRM (post NACT) :

Verified by :

Dr.Heba Hasan

Printing Date : 05/05/2025 09:42:31

Page 1 of 2

Printing By : marwa mohamed youss



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / منى على صباح عمير
Patient No. : 28804032101507
Path No. : 1415/25
Sex : Female
Age : 36 Y
Referring Doc :

Request No. : 13008436
Request Date : 20/04/2025
Verification Date : 05/05/2025
Financial Category : اورام دار السلام - ميلد
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- No remnants of tumor tissue
- Free margins
 - Free nipple and areola
 - Free L.Ns (0/4)
 - Pathologic complete response
 - Stage : ypT0 , ypN0

Signature
Prof. Dr. Gamal Abdelatty
Dr.Menan Hosam

Verified by :
Dr.Heba Hasan

Printing Date : 08/05/2025 09:42:31

Page 2 of 2

Printing By : marwa mohamed younes



شقة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Mona Ali Sabah Omaira	Date : 02/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 28804032101507	Pat. ID: 80354

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Heterogenous dense fibro glandular density (ACR C)
- Right UOQ subtle architectural distortion with underlying suspicious microcalcifications.
- Left benign macrocalcifications
- No left suspicious micro calcifications or architectural distortion detected.

Complementary ultrasound :

- Right breast multiple irregular heterogenous hypoechoic solid mass lesions with pathological intervening parenchyma seen from 8o'clock to 10o'clock, are seen with highly vascularity within, are seen from skin about 2mm and from nipple about 8mm. the largest mass lesion seen at 10o'clock zone C measuring about 20x26mm.
- No left cystic or solid mass lesions.
- Normal left skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Multiple right pathological axillary enlarged LNs with diffuse cortical thickening and effaced fatty hilum, the largest one measuring about 25x14mm seen at level I (more than 3 nodes).
- Free supra and infra clavicular lymph nodes.
- Nonspecific left axillary LN.

Opinion:

- Right UOQ multifocal malignant looking lesions with ipsilateral pathological axillary lymph nodes (more than 3 nodes) at level I for histopathological correlation (BIRADS 5)
- Benign findings of left breast (BIRADS 2).

Signature Dr. Nehal Hamdy

02/10/2024 1:36 PM



Patient Name: Mona Ali Sabah Omaira	Date : 28/10/2024
Exam : C.T CHEST	Age : 36 Year(s)
Medical ID: 3703/2024	Pat. ID: 80354

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of right breast cancer.

I Chest:

- Multiple enlarged axillary level I, II & III lymph nodes, the largest at level I measures about 1.5 x 2.3 cm.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Enlarged liver showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenal glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- Reported case of right breast cancer revealed no sizable pulmonary, hepatic or osseous deposits on non-contrast basis.

Dr. Ali Elsisy, MSc.

Prof. Menna Talla Hanafy, M.D.



Patient Name: Mona Ali Sabah Omalra	Date : 28/10/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 36 Year(s)
Medical ID: 3703/2024	Pat. ID: 80354

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (14cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Unremarkable study

Signature Dr. Nermien Ramadan

28/10/2024 1:22 PM



الكلية الطبية
مركز الدراسات والبحوث
مؤسسة ناصر
Nasser Institute Hospital

Nuclear Medicine Unit

Patient Name: Mona Ali Sabah

Date: 17/11/2024

Referring physician: Prof.

^{99m}Tc -MDP BONE SCAN

Desired Information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- No scintigraphic evidence of bony metastases.
- Heterogenous abnormal radiotracer uptake is seen at the spine, more determined at mid dorsal, upper lumbar vertebrae likely of ~~Spondylodegenerative changes~~,
- Mild osteoarthritic changes are seen at both shoulders, both knees.
- Both kidneys are normally seen.

Conclusion:

No definite hot spot that could account for osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR. Sherin Mahdy, M.D

DR. Mohamed Emam

Patient Name: Mona Ali Sabah Omaira

Exam : ultrasound TRANS VAGINAL

Medical ID:

Date : 08/09/2025

Age : 37 Year(s)

Pat. ID: 80354

Trans-vaginal ultrasound scanning revealed: -

- Uterus: -
 - AVF, average size uterus measuring about: 8x4x4cm in its longitudinal, transverse & AP diameters respectively.
 - The myometrium shows homogenous texture with no focal lesion
 - The endometrial line is 3mm. IUCD is in place
 - Multiple myometrial & parametrial varicosities
- Ovaries: -
 - Both ovaries are average size with no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Signature Dr . Nermien Ramadan
08/09/2025 2:02 PM



Patient's Name: Mona Ali Sabah Ameera
Date: 2026-08-15
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST-CONTRAST CT EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS**

CHEST:

- Evidence of right modified radical mastectomy.
- Subpleural reticulations in the right upper and middle lobes, likely representing post-radiation sequelae.
- No pulmonary nodules, masses, or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymphadenopathy.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

ABDOMEN AND PELVIS:

- Enlarged liver with homogenous parenchymal attenuation. No focal hepatic lesion or biliary dilatation.
- Both kidneys normal in site, size, and morphology. No stones or back pressure changes.
- Normal CT appearance of spleen, pancreas, adrenal glands, aorta, and IVC.
- Urinary bladder unremarkable.
- IUD noted within the uterus.
- No abdominal or pelvic masses, collections, or lymphadenopathy.
- No ascites.

OPINION: CT study shows:

- *No pulmonary or hepatic metastatic lesions.*

Dr.sherif Wagdy

Patient's Name: Mona Ali Sabah Ameera
Date: 2026-08-15
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST-CONTRAST CT EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS**

CHEST:

- Evidence of right modified radical mastectomy.
- Subpleural reticulations in the right upper and middle lobes, likely representing post-radiation sequelae.
- No pulmonary nodules, masses, or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymphadenopathy.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

ABDOMEN AND PELVIS:

- Enlarged liver with homogenous parenchymal attenuation. No focal hepatic lesion or biliary dilatation.
- Both kidneys normal in site, size, and morphology. No stones or back pressure changes.
- Normal CT appearance of spleen, pancreas, adrenal glands, aorta, and IVC.
- Urinary bladder unremarkable.
- IUD noted within the uterus.
- No abdominal or pelvic masses, collections, or lymphadenopathy.
- No ascites.

OPINION: CT study shows:

- *No pulmonary or hepatic metastatic lesions.*

Dr.sherif Wagdy

Patient's Name: Mona Ali Sabah Ameera
Date: 2026-08-15
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST-CONTRAST CT EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS**

CHEST:

- Evidence of right modified radical mastectomy.
- Subpleural reticulations in the right upper and middle lobes, likely representing post-radiation sequelae.
- No pulmonary nodules, masses, or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymphadenopathy.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

ABDOMEN AND PELVIS:

- Enlarged liver with homogenous parenchymal attenuation. No focal hepatic lesion or biliary dilatation.
- Both kidneys normal in site, size, and morphology. No stones or back pressure changes.
- Normal CT appearance of spleen, pancreas, adrenal glands, aorta, and IVC.
- Urinary bladder unremarkable.
- IUD noted within the uterus.
- No abdominal or pelvic masses, collections, or lymphadenopathy.
- No ascites.

OPINION: CT study shows:

- *No pulmonary or hepatic metastatic lesions.*

Dr.sherif Wagdy